

URIMIX ORAL SOLUTION

COMPOSITION

Each 10ml contains:

Potassium citrate:	3g
Citric Acid Monohydrate :	0.5g

Preservative:

Sodium Benzoate	10mg
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PRODUCT DESCRIPTION

Clear to slight haze liquid having lemon flavor.

PHARMACODYNAMICS

Potassium citrate and citric acid solutions are systematic and urinary alkalinizers thereby providing symptomatic relief of dysuria.

PHARMACOKINETICS

Potassium Citrate is absorbed and the citrate is metabolised to bicarbonate. Citric acid is metabolised to carbon dioxide and water. Oxidation is virtually complete with less than 5% of citrate being excreted unchanged in the urine.

INDICATION

For systemic or urine alkalinisation

For the symptomatic relief of dysuria associated with mild urinary tract infections, especially cystitis.

RECOMMENDED DOSAGE

Adult: 15-30 ml well diluted with water.

Children

Up to 1 year: 2.5 ml 3 times daily;

1 to 5 years: 5 ml 3 times daily;

6 to 12 years: 10 ml 3 times daily.

To be taken well diluted with water, after meals and at bedtime.

ROUTE OF ADMINISTRATION

Oral

CONTRAINDICATIONS

Hypersensitivity to Potassium Citrate or Citric Acid or any of the other ingredients/components of the product. Severe renal impairment with oliguria or azotemia, untreated Addison's disease, adynamia episodica hereditaria, acute dehydration, heat cramps, anuria, severe myocardial damage, and hyperkalaemia from any cause.

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WARNING AND PRECAUTIONS

Large doses may cause hyperkalaemia and alkalosis, especially in the presence of renal disease. Concurrent administration of potassium-containing medication, potassium-sparing diuretics, angiotensin-converting enzyme (ACE) inhibitors, or cardiac glycosides may lead to toxicity.

Should be used with caution by patients with low urinary output unless under the supervision of a physician. As with all liquids containing a high concentration of potassium, patients should be directed to dilute adequately with water to minimize the possibility of gastrointestinal injury associated with the oral ingestion of concentrated potassium salt preparations; and preferably, to take each dose after meals to avoid saline laxative effect.

DRUG INTERACTIONS

Concurrent administration of potassium-containing drugs, potassium-sparing diuretics or other drugs that increase potassium levels (e.g. ACE inhibitors, ciclosporin, aliskiren) may lead to hyperkalaemia. May interact with cardiac glycosides. Citrates alkalinise the urine and thus may alter the urinary excretion of a number of drugs. This may lead to increased renal clearance of acidic drugs, such as salicylates, tetracyclines and barbiturates, and prolongation of the half-life of basic drugs, such as sympathomimetics and stimulants. Particularly noteworthy is the diminished anti-bacterial activity of nitrofurantoin and methenamine.

PREGNANCY AND LACTATION

No adverse effects are anticipated at recommended doses when used for initial symptomatic relief only. Treatment with potassium citrate mixture is adjunctive and secondary to anti-bacterial treatment of urinary tract infection.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES

No effect on mental alertness.

ADVERSE EFFECTS

Abdominal discomfort, vomiting, diarrhoea, loose bowel movements, nausea, gastrointestinal disturbances and mild diuresis. High levels of potassium in blood may cause muscle weakness, irregular heart action and paralysis.

Potentially Fatal: Hyperkalaemia.

OVERDOSE AND TREATMENT

Symptoms:

Overdosage is accompanied by nausea, vomiting, abdominal pain and symptoms due to hyperkalaemia and metabolic acidosis. Fluid and electrolyte balance together with ECG should be closely monitored.

Hyperkalaemia is manifested by increased serum potassium concentration and ECG changes, while late manifestations include muscle paralysis and cardiovascular (CV) collapse from cardiac arrest.

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Treatment:

Symptomatic and supportive. Moderate to severe hyperkalaemia is a medical emergency requiring prompt correction. Eliminate medications containing potassium, agents with potassium sparing properties (e.g. potassium-sparing diuretics, ACE inhibitors, NSAIDs) and food with high potassium content (e.g. almonds, beans, milk, salmon). May give intravascular (IV) calcium gluconate if the patient is at no risk or low risk of developing digitalis toxicity. Administer IV dextrose 10% containing 10-20 U of crystalline insulin per 1,000 mL at a rate of 300-500 mL/hr. Acidosis may be corrected with IV Na bicarbonate. May perform haemodialysis or peritoneal dialysis.

STORAGE CONDITIONS

Store below 30°C.

Protect from light.

PRESENTATION

HDPE Bottle of 60ml and 120ml

MANUFACTURER AND PRODUCT REGISTRATION HOLDER

Noripharma Sdn. Bhd. 200701034604 (792633-A)

Lot 5030 Jalan Teratai,
5½ Mile Off Jalan Meru,
41050 Klang, Selangor, Malaysia

DATE OF REVISION

September 2025